جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : شرين عبدالمحسن ابراهيم عبدالمحسن
المحافظة : الجيزه **الحي / المدينة :** البدرشين **العنوان :** ....
رقم البطاقة : 28001052101303 **رقم التليفون :** 01116214780 **تاريخ تسجيل الإستمارة :** 13-10-2026
العمر(تاريخ الميلاد) : 05-01-1980 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
التوقيع:(----------)

الإخصائي الاجتماعي
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: شرين عبدالمحسن ابراهيم عبدالمحسن
National ID: 28001052101303

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





Dmg

كيادى البودا

نومينا

تعسيس

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

05/22/2026

MDT FORM

Name:	Shirin Abd Almohsin Ebrahim	MRN:	49
Age:	46 Y	ID:	28001052101303
Menopausal Sta		Phone:	01125253242
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	-ve	Reason Of MDT:	metastic

Clinical Data:

BX:

ER: 8/8 PR: 8/8 HER2: -ve KI67:

Sonomammography:

Mets Workup:

46 years old female pt with metastatic breast cancer to bone RT BREAST CT4DN1M1 IDCGII ER8/8 PR 8/8 HER2-VE ZOLADEX+AI 9/2024 TILL 8/2025 PD ON IBRANCE +FASLODEX CT CAP 9/2/2026 NEWLY DEVELOPED FEW BILATERAL SCATTERED PULMONARY NODULES LARGEST SEEN 5.7 MM AFTER CONSULTING DR KEUANY NO SIGNIFICANT PROGRESSION SD OF REST OF STUDY BONE SCAN 24/1/2026 SD.
MRI brain dated: 2/5/2026: SOL for contrast enhancing study, signs of increased intracranial tension

6/2026 CT: pd +newly developed pulmonary nodules
6/2026 mri brain: edema +mass suggesting Pach meningeal deposit
clinically: pd
for mdt

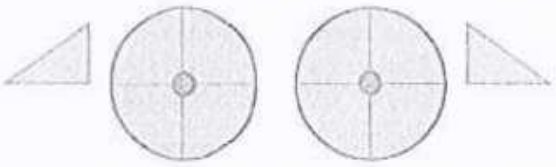
History:

Recommendations:

MDT Outcome & Plan

In view of straight progression on Palbociclib, and symptomatic meningeal deposits....for:
1.WCI 30 Gy/5# with dehydrating measures.
2.Shift to Xeloda thereafter

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
[illegible]					

22/6/26

MDT Form
[[DOCTYPE.MDT_FORM]]

Name: شرين عبد المحسن ابراهيم	
MR. : 49	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : تاريخ وساعة الخروج :

وبعد الكشف والفحوصات والمتابعة وجد انه

FOR IBRANCE 125 1*1*21 FASLODEX 500 MG CALCIUM ONE ALPHA NEXT ZOLADEX 10.8 SC AT 7/4/2026 ZOMETA 4 MG AT 4/2/2026 (PHARMACY) FOR BONE SCAN +CT ACP

46 years old femae pt witj metstatic breast cancer to bone RT BREAST CT4DN1M1 IDCGII ER8/8 PR 8/8 HER2-VE ZOLADEX+AI 9/2024 TILL 8/2025 PD ON IBRANCE +FASLODEX CT CAP 9/2/2026 NEWLY DEVELOPED FEW BILATERAL SCATTERES PILMONARY NODULES LARGEST SEEN 5.7 MM AFTER CONSULTING DR KELANY NO SIGNIFICANT PROGRESSION SD OF REST OF STUDY BONE SCAN 24/1/2026 SD POSPONED IBRANCE TLC 2.32 NEUT 0.87 ALT 52 AST 42 FASLODEX 10.8 SC NEXT ZOLADEX10.8 SC AT 7/4/2026

Today for faslodex 500mg im , zoladex (pharmacy once approval) mDT >> still pending

MRI brain dated : 2/5/2026 : SOL for contrast enhancing study , signs of increased intracranial tension for new MRI brain w contrast new CTCAP for MDT (requested)

MDT for: 1-WCI, dehydrating measures 2- start xeloda approval sent

pt finished WCI from 7/7 till 22/7 for xeloda wt 75 ht 168 4x3x14 xeloda + h blocker + antiemetic + zometa, cbc pending, kft and ca accepted +zoladex

pt came ps3 on wheel chair for c2 xeloda 4+3 no reported toxicities No CBC LFTs to dispense after labs labs ast 121 / 88 no dose adjustment for liver support last evaluation 6/2024 for n ew after 4 months has LL edema : for duplex

LL duplex 24/8 > No evidence of DVT. C3 Xeloda 4+3 no reported toxicity, labs accepted liver enzymes improving , for evaluation CT CAP after finisihing C3

تحريرا في ,

التاريخ : 19-09-2026 الوقت :

الطبيب المعالج : ايه مدحت بدوي محمد

يعتمد ,

Patient's Name:	SHEREEN ABD EL MOHSEN IBRAHIM
Date:	2026-05-02
Referred by:	Prof.Dr. MRI BRAIN
Hospital:	Goustave Roussey International Egypt

MRI OF THE BRAIN

Technique :

Axial T1, T2 ,FLAIR,
Coronal T2
Sagittal T2

Findings:

- Right temporal area of altered MR signal eliciting low T1 and bright T2 and FLAIR WI signal with surrounding edema. It is seen causing mild mass effect in the form of effacement of the adjacent cortical sulci.
- Normal size and configuration of the supra and infra tentorial ventricular system
- No midline shift or deformity
- No cerebellar or brain stem areas of altered MR signal.
- No extra axial collections.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.
- Partial empty sella.
- Prominent peri-optic CSF Spaces bilaterally.
- Bilateral oto-mastoiditis.
- Right maxillary sinusitis.

OPINION:

- ***Right temporal area of altered MR signal ? SOL for contrast enhanced study.***
- ***Partial empty sella with prominent peri-optic CSF spaces bilaterally raising the possibility of benign increased intracranial tension for clinical correlation.***
- ***Bilateral oto-mastoiditis.***

Dr.sherif Wagdy

M.R.I
[[DOCTYPE:MRI]]

Patient's Name:	Sherin Abdul Mohsen Ibrahim Abdul Mohsen
Date:	2026-06-10
Referred by:	Prof.Dr. no doctor
Hospital:	Gustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

A known case of right breast cancer, compared to the previous study dated 9-2-2026, the current study revealed:

I- CHEST:

- Rather stationary course regarding the size of the previously seen right breast lesion measuring about 3.6x3.1 cm with overlying skin thickening measures and prominent ipsilateral axillar lymph nodes, the largest measures about 12 x11mm.
- Mild progressive course regarding the number of the bilateral scattered pulmonary nodules, with the largest right upper lobe measuring about 5.5 mm.
- Still noted bilateral basal atelectatic bands.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Mildly enlarged liver showing homogenous fatty parenchymal attenuation. Multiple bilobar newly developed hypodense hepatic focal lesions largest measuring 65 x 33 mm at segment II. No dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No significant time interval changes the previously noted mesenteric area of fat stranding and multiple subcentimeteric mesenteric lymph nodes likely mesenteric panniculitis.

CT Scan
[[DOCTYPE CT_SCAN]]

Patient's Name:	Sherin Abdul Mohsen Ibrahim Abdul Mohsen
Date:	2026-06-10
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

- No abdominal or pelvic lymphadenopathy.
- No ascites.
- The examined skeleton showed still noted multiple axial and appendicular sclerotic lesions.

Opinion:

- **Progressive course regarding the number of the bilateral scattered pulmonary nodules as well as the multiple newly developed bilobar hepatic focal lesions.**
- Rather stationary course regarding the rest of the examination.

Dr.sherif Wagdy

CT Scan
[[DOCTYPE CT_SCAN]]

Name: Sherein Abdelmohsen
Date : 01/06/2026
Ref. Physician:

WITH CONTRAST MRI OF THE BRAIN

Technique:
Variable pulse sequences in different planes.

MRI Findings:

- Right temporal occipitoparietal vasogenic edema is noted with the related cortical thickening and abnormal signal. Associated mass-effect is noted with mild effacement of the right lateral ventricle and midline structural shift about 6 mm to the left side.
- Right cerebellar hemisphere vasogenic edema signal is also noted.
- Diffuse convexity meningeal thickening and diffuse smooth enhancement is noted with nodular enhancement component at the right temporal occipital and right cerebellar region.
- Diffuse altered bone marrow signal of the calvarium showing a sclerosis texture.
- No gross vascular abnormality.
- Normal brain stem & cervico – medullary junction.
- Normal sellar region.

Opinion:

- ***Known patient with metastatic cancer breast showing diffuse meningeal thickening and enhancement more on the right side with the related intra-axial edema and associated mass-effect as described suggesting pachymeningeal deposits.***
- ***Diffuse altered bone marrow signal of the scalp denoting infiltrative lesion.***

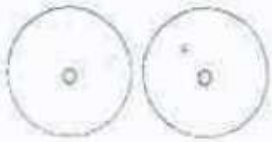
Best regards
Dr/Mohamed Abdelrazek, MD

Palestinian Program for Women's Health
Ministry of Health and Population

MDT form

A-Patient ID:
1- Name : نسرين عبدالمحسن ابراهيم عبدالمحسن Sex: Female
3- Medical number: 83220136
Old Mrd : 2314-2024

2-ID number: 28001052101303 Patient Age : 45 Y



Family history	-ve	Phone		BMI		REASON OF MDT	
marital status		NRS Score		GROUP		Date of MDT	2025-04-22
Co morbidity	-ve	Menopausal status		Initial STAGE			

Clirical data

Breast Imaging :	Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	8/8	0		16

T	N	M
4d	1	1(bone)

Sonomammography

PATIENT HISTORY

bone scan: equivocal bone Mets

PT Received 1st Line AI + ZOLADEX WITH PD

CT CAP : PD OF BONE METS, MILD PROGRESSION OF BREAST MASS

MDT OUTCOME & PLAN

IBRANCE + FASOLDEX + ZOMETA Q MONTHLY

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الأستاذه / شريف محسن ابراهيم عبد المحسن	Request No.	: 13000059
Patient No.	: 28001052101303	Request Date	: 02/07/2024
Path No.	: 2200/24	Verification Date	: 16/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 44 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast swelling.
Nature of Specimen: Tru-cut biopsy, collected at 1/7/2024.

Gross Examination:
- Four grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* Ductal carcinoma in situ: Focally positive.
* Perineural invasion: Negative.
* Angioinvasion: Negative.
* Microcalcifications: Negative.

Diagnosis:
Right breast swelling, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, with focal DCIS.
For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by:

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / شرين عبد المحسن ابراهيم عبد المحسن	Request No.	: 13000059
Patient No.	: 28001052101303	Request Date	: 02/07/2024
Lab No.	: 2200/24	Verification Date	: 21/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 44 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Regist	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 1/7/2024.
Technique:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
IPT (Positive control showed appropriate reactivity):
ER: Strong nuclear reaction in 70 % of tumor cells (Allred score 8/8).
PR: Strong nuclear reaction in 70 % of tumor cells (Allred score 8/8).
Her2: Negative membrane reaction in tumor cells (score 0).
Ki67: 16 % nuclear reaction of tumor cells.

Conclusion:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, with focal DCIS.
IPT:
ER: Positive (score 8/8).
PR: Positive (score 8/8).
Her2: Negative (score 0).
Ki67: Positive in 16 % of tumor cells.

Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (٩٥٦) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 956 - Nile Corniche - Misr - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

atient Name	الأستاذه / شرين عبد المحسن ابراهيم عبد المحسن	Request No.	13000059
atient No.	28001052101303	Request Date	02/07/2024
th No.	2200/24	Verification Date	21/07/2024
ex	Female	Financial Category	اورنم دار السلام - مباشره
ge	44 Y	Department	Out Patient
eferreing Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, rmediate; and 3, strong). Total score from 0 to 8.
coring system: "Her2"
ference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining served or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely ceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane ining that is incomplete and/or weak/moderate and within > 10%of the invasive tumor cells or nplete and circumferential membrane staining that is intense and within < 10% of the invasive tumor ls. IHC 3+: based on circumferential membrane staining that is incomplete, intense > 10% of tumor

Signature
Prof. Dr. Lubna Omar.
Dr. Toka Medhat.

Verified by :

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE IHC]]

Ministry of health
Central health laboratories
Pathology department

Report Number: m599

ImmunohistoChemistry report

المريض	: ريهام فاروق حسين	النوع	: Female
	: 43 Y	العنوان	: القاهرة
تاريخ أخذ العينة	:	تاريخ استلام العينة	: 20/03/2024 11:59
التحليل	: معهد ناصر	تاريخ التقرير	: 25/03/2024 09:57
	: معهد ناصر		

Primary Sample :

Paraffin Block

Test :

ER &PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

Sample received was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (1E2)Rabbit monoclonal primary antibody.
Sample recieved was treated by : -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX / XT .
Sample received was treated by :- CONFIRM anti Ki67 (30-9) Rabbeit monoclonal primary antibody.

Result :

ER: No nuclear staining of tumour cells (tiny fragment)
PR: No nuclear staining of tumour cells (tiny fragment)
HER2/neu protein over expression : strong continuous membrane staining of > 10 % of tumor cells. (tiny fragment)
Ki67: positive nuclear staining of < 20 % of tumour cells (tiny fragment)

Referal Lab :

Comment :

Conclusion :

ER: NEGATIVE
PR: NEGATIVE
HER2/NEU PROTEIN OVER EXPRESSION : POSITIVE (SCORE 3 +)
KI67: THE PROLIFERATION INDEX OF THE TUMOUR IS < 20 %

Pathologist

دالوزا ماهر

Lab Director: دالوزا ماهر

Ghamrawi Rehan Street Abdeen. Cairo
Tel: 27948544 -7953096 Fax (202)7948544
Mobile 01144297252
Email: pathlabayoul@yahoo.com
Website: http://www.healthlinks.net

This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بالعينة لمدة 30 يوم من تاريخ التقرير

immunohistochemistry
[[DOCTYPE IHC]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Sherine Abdel Mohsen Ibrahim Abdel Mohsen	Date : 04/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 46 Year(s)
Medical ID:	Pat. ID: 54790

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA. compared to the previous study dated 28/4/2025, the current study showed:

I- CHEST:

- No appreciable time interval changes regarding the size of the previously seen right breast lesion measuring about 36 x 31 mm as well as the previously seen skin thickening.
- Stationary course regarding the previously seen right axillary lymph node measuring 11 mm.
- Few newly developed tiny right upper and lower lobe pulmonary nodules largest measuring 3.8 mm at the right upper lobe.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Mildly enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Regressive course since last study regarding the previously seen mesenteric area of fat stranding and multiple subcentimeteric mesenteric lymph nodes.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- No appreciable time interval changes regarding the previously seen multiple axial and appendicular skeleton sclerotic bone lesions.

CT Scan
[[DOCTYPE CT_SCAN]]

Opinion:

- Progressive course regarding the few newly developed right upper and lower lobe pulmonary nodules.
- Regressive course regarding the previously seen mesenteric panniculitis.
- Stationary course regarding the rest of the study.

Dr. Sherif Wagdy

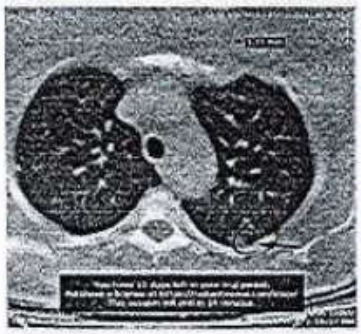
Dr. Sherif Wagdy

CT Scan
[[DOCTYPE CT_SCAN]]

Name: Shereen Abd El Mohsen Ibrahim.
Date: 19-07-2025.

Known case of right breast cancer on radiotherapy and hormonal therapy:
Compared to study dated 13-02-2025, Post contrast CT scan of the chest, abdomen and pelvis revealed

- **Stationary course as regards the size of the right breast retroareolar malignant mass, of speculated shape and now measuring about 46.9x 25.8 mm, (previous study=46x 25 mm), in axial cuts.**
- **The previously noted right axillary LNs now show single one measuring about 11 mm in diameter.**
- **Newly developed left upper lobar posterior segment subpleural nodule= 8 x 5 mm in axial diameters.**
- Clear rest of the left lung field as well as the whole right lung field.



- No gross hilar or mediastinal glandular enlargements.
- No pleural collections or masses seen.
- No gross cardiac abnormality.
- No pericardial collection seen.
- **Multiple extensive thoracic vertebral, sternal and bilateral scapular sclerotic lesions are seen, giving the bones a mosaic pattern, likely metastatic, with stationary course.**
  as compared to previous study done 13-02-2025.

CT scan of the abdomen and pelvis with oral and IV contrast revealed:-

- The Liver is **mildly enlarged in size with diffuse hypodense uniform** attenuation, no focal lesions seen in any phase, no intra or extra hepatic biliary radicle dilatation, patent main portal vein and its central branches.
- The spleen is average in size with no focal lesions.
- Normal CT appearance of the pancreas, kidneys, aorta & IVC.
- No suprarenal masses seen.
- No CT evidence of porta-hepatis or para-aortic lymph node enlargement.

CT Scan
[[DOCTYPE:CT_SCAN]]

- No ascites.
- Free flow of the oral contrast through different bowel segments with no obstruction seen.
- The urinary bladder shows no stones or masses.
- **Multiple extensive thoracic and lumbosacral vertebral sclerotic and lytic lesions are seen, giving the spine mosaic pattern, with bilateral femoral upper ends and pelvic bony lesions likely metastatic in nature, with stationary course as compared to previous study dated 13-02-2025.**
- Normal appearance of the rest of the pelvic organs.



Opinion:-

- Known case of breast cancer, on hormonal and chemotherapy in comparsion to study dated 13-02-2025 current study revealed:

- Newly developed left lung upper lobe subpleural nodule, likely metastatic

- Stationary course as regards the skeletal metastatic affection, of widespread bony metastatic disease.

- Stationary course as regards the size of the right breast rertroareolar mass.

- Regressive course as regards the number (remaining single LN), but not the size of the right axillary LN).

- Fatty hepatomegaly.

MUCH OBLIGED,

Dr. H,.. Al Enany,...FEBR

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Sherine Abdel Mohsen Ibrahim Abdel Mohsen	Date : 02/02/2025
Exam : MAMMOGRAPHY BOTH	Age : 45 Year(s)
Medical ID: 2314/2024	Pat. ID: 54790

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Right breast cancer for follow up

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

Right Breast:

Still noted partially obscured high density lesion is seen in the right UOQ and UIQ with adjacent spicules and skin dimpling / thickening covering an area measures 61 X 94 mm in CC view seen by U/S as 9 to 1 o'clock infiltrative mass lesion with parenchymal distortion with overlying skin edema and dimpling as well as suspected intraductal extension towards the nipple.

Regression of pathological enlarged level I and II axillary LN largest measures4 x 8 mm compared to 8 X 12 mm.

No pathological enlarged supraclavicular lymph nodes.

Left Breast:

(ACR B)

No suspicious micro calcifications or architectural distortion detected.

Complementary ultrasound no cystic or solid mass lesions.

Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.

Nonspecific axillary LN.

Opinion:

Status follow up showing stable right breast malignant multicentric process with regression of ipsilateral axillary lymphadenopathy (BIRADS 6).

Normal sonomammography of left breast (BIRADS 1).

Signature Dr. Wafaa Ali

02/02/2025 1:45 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Name : Sherin Abdul Mohsen Ibrahim Abdul Mohsen **Date : 29-09-2025 09:59**

Branch : Dokki **Medical No. : 213557**

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History

- A 46-year-old lady, with history of metastatic breast cancer and referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

Considering the prior bone scan dated 14/04/2025:

- Still seen active osseous lesions at skull, sternum, both shoulders, few bilateral ribs, multi-levels of the spine, pelvic bones and proximal both femora.
- No other abnormal active or cold focal bony lesions could be noted.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Metastatic osseous lesions with rather stationary course as descried.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine



جمهورية مصر العربية
بطاقة تحقيق الشخصية

شرين
عبدالمحسن ابراهيم عبدالمحسن
قرية مزغونة
مركز البدرشين - الجيزه

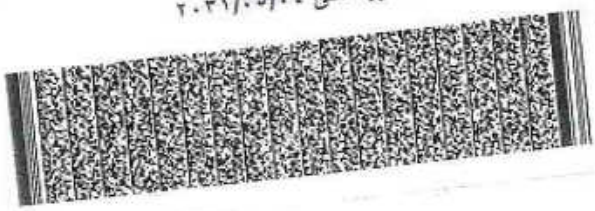
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٢٨٠٠١٠٥٢١٠١٣٠٣ ٢٠٢٤/٠٥
حاصلة على دبلوم صناعى

انثى مسلمة متزوجة
ايمن عبدالقادر ابراهيم النحاس
البطاقة سارية حتى ٢٠٣١/٠٥/٠٤



GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

السيد الاستاذ الدكتور / مدير عام المجالس الطبية

تحية طيبه وبعد ،،،

نفيد علم سيادتكم علما بأن السيد/ة : شرين عبدالمحسن ابراهيم عبدالمحسن

صاحب الرقم القومي : ٢٨٠١٠٥٢١٠١٢٠٣

رقم القرار : ٤١١٤٧٢١١

بانه لم يتم استخدام قرار نفقة الدولة ولم يتم انشاء اي فاتورة الكترونية للقرار المطلوب الغاءه

وهذا للعلم والاحاطه

ولسيادتكم جزيل الشكر ،،،

الموظف المختص نورهان رمضان

مدير الايرادات محمد فؤاد

مدير المستشفى

GRI-E
جوستاف روسي
جوستاف روسي - مصر

كود المدخل	كود المراجع	كود الطبيب
٢٨٦٦٤	١٤٧٤٣	٢٥٦٠٢

توصية المجلس الطبي المتخصص

رقم ٤١١٤٧٢١١ لسنة ٢٠٢٦
الرقم القومي ٢٨٠٠١٠٥٢١٠١٣٠٣

الادارة العامة للمجالس الطبية المتخصصة
- بعد الاطلاع علي تقرير الكشف الطبي والابحاث الخاصه بالمواطن المذكور ادناه
- ووفقا لما جاء بقرار وزير الصحه رقم ٤٠٨ لسنة ٢٠٠٩
- وعلي توصية المجلس الطبي المتخصص رقم ٤١٠١٨٧٤٠ لسنة ٢٠٢٦/٠٩/٠٣
- وعلي تقرير اللجنه الثلاثية الصادر من المستشفي بتاريخ ٢٠٢٦/٠٩/١٠
و المسجل على النظام بتاريخ ٢٠٢٦/٠٩/١٠
- اوصي المجلس الطبي المتخصص بجلسة ٢٠٢٦/٠٩/١٤

بالموافقة علي استكمال علاج المواطن / شرين عبدالمحسن ابراهيم عبدالمحسن من سرطان بالثدي (مبادرة) الصادر فى شأنه التوصية رقم ٤١٠١٨٧٤٠ بمستشفى جوستاف روسي انترناشيونال - مصر (EPE) * بقسم الاورام خارجى اشعه مقطعيه علي البطن والصدر بالصبغه (كود ١١١٢٧) .
وفقا لما جاء باسعار بروتوكولات العلاج علي نفقة الدولة وطبقا لكود الحالة المرضية

تحريرا فى ٢٠٢٦/٠٩/١٤

تعتبر التوصيه نافذه من تاريخ تحرير اللجنه الثلاثيه
لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض ومناظرة المريض بالمستشفي قبل الصرف
قيمة القرار مخصصة للعلاج ولا يجوز المطالبة بأي نفقات أخرى سواء للإنتقال أو أي إجراء خارج منطوق القرار

مراجعة ادارية

حنان اسماعيل . .

مراجعة طبية

د/ وسام محمود عبدالعزيز -

مدير عام

الإدارة العامة للمجالس الطبية المتخصصة

د/ محمد عقاد

جوستاف روسي انترناشيونال - مصر (EPE) *
طبع فى : ٢٠٢٦/٠٩/٢٣ ٦٧٨١٢٥٠٠